

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think>

Finding Reasoning Paths:

1. **Hyperactivity , impulsivity , difficulty concentrating , and frequent inattention** suggest Attention - Deficit /Hyperactivity Disorder (ADHD). Evaluate whether behavioral therapy (e.g ., summer camp programs for ADHD , incorporating physical activity) could address these symptoms .

2. **Chronic inattention and difficulty concentrating** may indicate a need for stimulant therapy (e.g ., methylphenidate , dextroamphetamine , amphetamine) as first-line treatment for ADHD .

3. **Inattentiveness and frequent inattention** could also be linked to other conditions like learning disabilities or emotional /behavioral disorders , but the constellation of symptoms strongly suggests ADHD .

4. **Intermittent episodes of acute otitis media** are unlikely to be directly related to the behavioral symptoms but may be relevant for broader health monitoring .

5. **Family history of major depressive disorder** does not directly explain the ADHD-like symptoms but may suggest the need for comorbid psychiatric support .

Reasoning Process :

1. The patient is a 5 -year -old boy presenting with symptoms of hyperactivity , impulsivity , inattentiveness , and difficulty concentrating . These symptoms are consistent with a diagnosis of **Attention - Deficit /Hyperactivity Disorder (ADHD)** . The symptoms include frequent inattention , impulsivity , hyperactivity , and disorganization .

2. ADHD is commonly treated with **behavioral therapy** , which includes non-pharmacological interventions such as education , behavioral modification , and behavioral programs . For school-age children with ADHD , programs like summer camp programs or in-class support have been shown to be effective in improving behavior and social interactions .

3. Medications such as **stimulants** (e.g ., methylphenidate , dextroamphetamine , amphetamine) are first-line treatments for ADHD and are typically considered after behavioral therapy has been tried , especially in children . However , stimulants are not the immediate next step in this case , as the primary treatment approach for ADHD in this age group often begins with behavioral interventions .

4. The patient's symptoms do not appear to be due to other conditions like autism spectrum disorder (ASD) or intellectual disability , as the symptoms are well-defined and align with ADHD criteria . Additionally , the absence of other significant abnormalities on physical examination and the absence of developmental delays further support a diagnosis of ADHD .

5. The **family history of major depressive disorder** in the mother and **Graves ' disease** in the father suggest a potential for mood or endocrine disorders , but these are unlikely to explain the acute behavioral symptoms . However , psychiatric comorbidities should be monitored and addressed as needed .

6. While hyperactivity and impulsivity may benefit from physical activity or exercise programs , these are adjunctive measures and not the primary treatment for ADHD . Behavioral therapy remains the cornerstone of treatment for ADHD , especially in young children .

Conclusion :

The most appropriate next step in treatment is **behavioral therapy** , such as a program for ADHD that incorporates physical activity and behavioral modification . This aligns with evidence-based guidelines for treating ADHD in children .

</think>

<answer>

Behavior therapy

</answer>

1.5

1.0

0.5

0.0

-0.5

-1.0

-1.5